Clinical Study Summary Report (CSSR)
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1. Administrative & Study Identification

Full Study Title: A Blinded, Randomised, Placebo-controlled Study to Investigate the Safety, Tolerability, and Pharmacokinetics of an Oral Suspension of AZD6793 Following Single and Multiple Ascending Doses in Healthy Subjects, an Open-label Study to Assess the Relative Bioavailability and Food Effect of a Tablet Formulation of AZD6793 in Healthy Subjects and a Blinded, Randomised, Placebo-controlled Study to Investigate the Safety, Tolerability, and Pharmacokinetics of a Tablet Formulation of AZD6793 in Patients With Chronic Obstructive Pulmonary Disease
Public Study Title: A Study to Investigate the Safety, Tolerability, and Pharmacokinetics (PK) of Oral AZD6793 in Healthy and Chronic Obstructive Pulmonary Disease Participants, to Assess the Relative Oral Bioavailability Between Two Formulations, and the Food Effect on the PK of AZD6793 Compared to Fasting State **Short Title/Acronym:** Safety, Tolerability, and PK of Oral AZD6793 in Healthy and COPD Participants **Protocol Number:** D7860C00001 **NCT Number:** NCT05662033 **EudraCT Number:** 2022-002951-21 **Sponsor Name:** AstraZeneca **Investigational Product:** AZD6793 (Oral suspension and Tablet formulations) **Phase of Development:** Phase 1 **Medical Monitor:** AstraZeneca Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety and tolerability of AZD6793 suspension following oral administration of Single Ascending Dose (SAD) and Multiple Ascending Dose (MAD) in healthy participants, as well as a tablet formulation in patients with moderate to severe COPD. **Secondary Objectives:** To evaluate the pharmacokinetics (PK) parameters of AZD6793. To assess the relative oral bioavailability between two formulations and the food effect on the PK of AZD6793 compared to the fasting state.

2.2 Background & Rationale AZD6793 is a novel oral inhibitor of Interleukin-1 receptor-associated kinase 4 (IRAK4). IRAK4 targets the signaling of Interleukins and toll-like receptors, which heavily contribute to lung inflammation in chronic obstructive pulmonary disease (COPD). This first-in-human study evaluates the safety, tolerability, PK, and relative bioavailability of AZD6793 in healthy volunteers and establishes preliminary safety and target engagement parameters in patients with moderate-to-severe COPD.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Primary Purpose:** Treatment **Intervention Model:** Sequential Assignment **Allocation:** Randomized (3:1 ratio of AZD6793 to Placebo) **Control Type:** Placebo-controlled (Parts 1, 2, and 4) **Masking / Blinding:** Blinded (Parts 1, 2, and 4), Open-label for the bioavailability and food effect cohort (Part 3)

3.2 Planned Enrollment & Duration Actual Enrollment (N): 93 participants (SAD cohort $n=32$; MAD cohort $n=33$; Bioavailability cohort $n=13$; COPD cohort $n=15$ [10 AZD6793, 5 Placebo]) **Number of Sites:** Selected clinical pharmacology units (1 center for Parts 1-3; 2 centers for Part 4) **Study Start Date:** 05-Dec-2022 **Primary Completion Date:** 29-Oct-2024 **Study Completion Date:** 29-Oct-2024 **Study Status:** Completed

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Healthy male or female participants aged 18 to 55 years with suitable veins for cannulation, having a Body Mass Index (BMI) between 18 and 30 kg/m^2 and weighing at least 50 kg (Parts 1-3). 2. Male and/or female participants aged 40 to 80 years inclusive with moderate to severe COPD (Part 4). 3. For COPD patients: Documented stable inhaled treatment regimen of dual therapy or triple therapy for ≥ 3 months prior to Screening. 4. Females of childbearing potential must have a negative pregnancy test and agree to use an approved method of highly effective contraception.

4.2 Exclusion Criteria 1. History or presence of gastrointestinal, hepatic, renal, or pancreatic disease, or history of chronic haematologic disease. 2. Any clinically important abnormalities in resting Electrocardiogram (ECG) or clinical chemistry. 3. Positive result on Screening for serum Hepatitis B (HBsAg), Hepatitis C antibody, Human Immunodeficiency Virus (HIV), or QuantiFERON® Tuberculosis (TB) test. 4. Current smokers or those who have smoked/used nicotine products (including e-cigarettes) within the previous 3 months, or known/suspected history of alcohol and drug abuse in the last year (applicable to healthy cohorts).

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Single Ascending Doses (SAD) (Part 1), Multiple Ascending Doses (MAD) for 7 days (Part 2), single doses for bioavailability/food effect (Part 3), and up to 28 days of dosing for COPD patients (Part 4). **Route of Administration:** Oral (Suspension and Film-coated Tablets) **Duration of Treatment:** Ranges from 1 day (SAD/bioavailability) to 7 days (MAD) and up to 28 days (COPD cohort).

5.2 Concomitant & Prohibited Therapy Permitted: Stable inhaled treatment regimens of dual or triple therapy for COPD patients. **Prohibited:** Drugs with a similar chemical structure or class to AZD6793; therapies introduced for clinically important illnesses within 4 weeks prior to first IMP administration.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -35 to -2	Informed Consent, Medical History, ECG, Safety Labs (TB, HIV, Hepatitis)
Admission/Baseline	Day -1	Admission to the Clinical Unit, Baseline Assessments, Pregnancy Testing
Treatment	Day 1 to 28	Randomization, Dosing of AZD6793 or Placebo, PK Blood Sampling, Safety Monitoring
Follow-up Visit	6 $\pm$ 2 days post-dose	Final Vital Signs, Exit Interview, Safety Assessments

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7. Outcome Measures & Findings (Endpoints)

7.1 Primary Endpoint Definition: Safety and tolerability evaluated by the incidence of Adverse Events (AEs) and Serious Adverse Events (SAEs). **Measurement Method:** Continuous clinical monitoring, 12-lead ECG, and laboratory analyses throughout the dosing and follow-up periods.

7.2 Secondary & Exploratory Endpoints Secondary (PK Parameters): Evaluated maximum observed concentration (C_{max}), area under the concentration-time curve (AUC), t_{max}, and terminal half-life. *Findings:* AZD6793 demonstrated linear PK with a mean terminal half-life of 11-15 hours. Formulation and food effects on AZD6793 exposure were modest (t_{max} of 4 hours in the fed state versus 2 hours in the fasted state). **Exploratory/PD (Target Engagement):** Assessed by measuring the inhibition of R848-induced cytokine release in whole blood ex vivo. *Findings:* Target engagement was confirmed in all healthy volunteer MAD cohorts and the patient cohort by a $\ge 75\%$ mean reduction in the release of interleukin-6 (IL-6) and tumor necrosis factor-α (TNF-α) from blood.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard clinical collection. Most AEs across all cohorts were mild, with the most common AE reported being headache. No severe AEs occurred during the study duration. **Serious Adverse Events (SAEs):** Evaluated continuously with standard 24-hour expedited reporting protocols. No Serious Adverse Events (SAEs) occurred in any cohort. **Data Monitoring Committee (DMC):** Internal safety review committee employed for staggered dose escalation decisions between cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Analysis Set for all AE/SAE evaluations (all participants receiving ≥ 1 dose of IMP). PK Analysis Set for individuals with evaluable PK data. **Sample Size Justification:** The $N=93$ sample size across all 4 parts was deemed appropriate for early Phase 1 safety, tolerability, PK profiling, and target engagement objectives. **Handling of Missing Data:** Standard Phase 1 PK imputation rules; values below the lower limit of quantification were set to zero for concentration summaries.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Intensive on-site monitoring during residential clinical unit periods to capture rigorous PK/PD and safety data. **Audit Trail:** Robust eCRF locking mechanisms and data clarification procedures applicable to early-phase clinical pharmacology testing.

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11. Study Identifiers & Governance

Primary ID: D7860C00001 **Registry Number:** NCT05662033 **EudraCT Number**: 2022-002951-21 **Sponsor/Lead Organization:** AstraZeneca **Public Study Title:** A Study to Investigate the Safety, Tolerability, and Pharmacokinetics (PK) of Oral AZD6793 in Healthy and Chronic Obstructive Pulmonary Disease Participants, to Assess the Relative Oral Bioavailability Between Two Formulations, and the Food Effect on the PK of AZD6793 Compared to Fasting State **Official Regulatory Title:** A Blinded, Randomised, Placebo-controlled Study to Investigate the Safety, Tolerability, and Pharmacokinetics of an Oral Suspension of AZD6793 Following Single and Multiple Ascending Doses in Healthy Subjects, an Open-label Study to Assess the Relative Bioavailability and Food Effect of a Tablet Formulation of AZD6793 in Healthy Subjects and a Blinded, Randomised, Placebo-controlled Study to Investigate the Safety, Tolerability, and Pharmacokinetics of a Tablet Formulation of AZD6793 in Patients With Chronic Obstructive Pulmonary Disease

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12. Clinical Framework (COPD Specific)

Primary Condition: Inflammatory diseases / Chronic Obstructive Pulmonary Disease (COPD) & Healthy Volunteers **Diagnosis Threshold:** Clinically diagnosed moderate to severe COPD **Medical Methodology:** IRAK4 Inhibition on top of Standard of Care (stable dual/triple inhaled therapy) **Optimization Target:** Reduction in Interleukin and toll-like receptor signaling (IL-6 / TNF-α cytokine release) to mitigate lung inflammation

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13. Study Procedures & Methodology

Management Pathway: Intensive Phase 1 residential clinical pharmacology unit protocols. **Education Protocol (HCP):** Dose escalation, stringent stopping criteria, and rigorous PK sample handling. **Education Protocol (Patient):** Fasting vs. High Fat High Calorie (HFHC) meal instructions, continuous confinement restrictions. **Quality Audit Mechanism:** Phase 1 clinic standard operating procedures and source data verification.

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14. Observation & Follow-Up Schedule

Total Duration: Approximately 4 to 8 weeks (including a screening period up to 35 days, treatment periods up to 28 days, and follow-up). **Onsite Visit Cadence:** Confinement to the clinical unit starting on Day -1 through required intensive sampling periods, followed by post-discharge visits. **Remote Monitoring Frequency:** N/A (Predominantly confined residential clinical assessment). **Checklist Review Interval:** Daily during clinical unit confinement for safety, AE check, and IMP administration.

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	**[DD-MMM-YYYY]**
Clinical Operations Lead	**[Name]**	___	***[DD-MMM-YYYY]***
Lead Statistician	***[Name]***	_____	[DD-MMM-YYYY]